A Multiplex Framework for Responsible AI-Augmented Mental Healthcare

Integrating Traditional Islamically-Integrated Psychotherapy with Ethical AI

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The AI Disruption: Promise and Peril

The Mental Health Crisis

- 1 in 8 people live with a mental disorder
- 75% in LMICs receive no treatment
- Africa: 0.05 psychiatrists per 100,000

AI Has Already Arrived

- ChatGPT: 100M users in 2 months
- 25% of UK GPs using LLMs (2025)
- Patients not waiting for permission

✦ The Promise

- Democratize access to knowledge
- Scale therapeutic support
- Reduce stigma barriers
- Empower patient voice

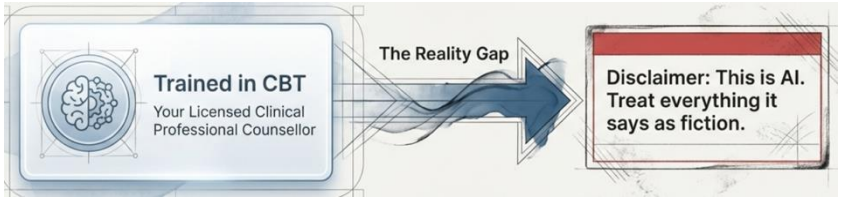
⚠ The Peril

- 15 ethical violations documented *(Iftikhar et al.)*
- Deceptive empathy: "I hear you"
- Cultural decontextualization
- Convergent solutions for divergent problems

Iftikhar, Z., Xiao, A., Ransom, S., Huang, J., and Suresh, H. (2025). *How LLM counselors violate ethical standards in mental health practice: A practitioner-informed framework*. In Proceedings of the AAAI/ACM Conference on AI, Ethics, and Society, volume 8, pages 1311–1323.

Pitfalls of contemporary frontier AI models

ABDICATION OF RESPONSIBILITY





LACK OF CRISIS MANAGEMENT



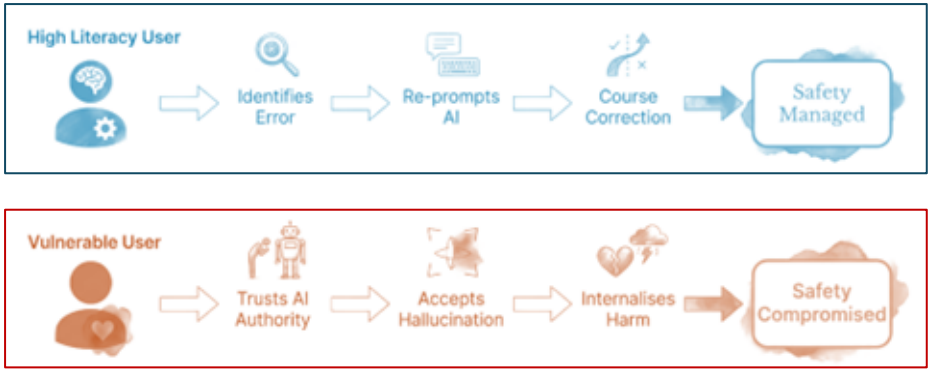
SYCHOPHANCY
Validating harm

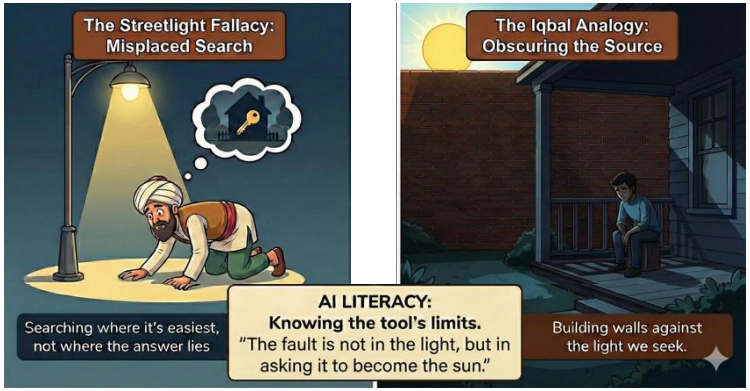


DECEPTIVE EMPATHY

These models show a strong Western-centrism and a religious bias

The Emotional Literacy Gap & *Deeper Questions*





The Consequence:
Those most in need of help are the most likely to accept harmful, hallucinated, or invalidating advice.

"Mental health" requires us to define:
what a mind is,
what health is,
and whose mind is it?

Yusuf, A., & al-Akiti, A. (2022). *Where the Two Oceans Meet: The Theology of Islam and the Philosophy of Psychiatric Medicine in Exploring the Human Self*. In Islam and Biomedicine (pp. 135-182). Cham: Springer International Publishing.

Two Answers to "What Is a Human Being?"

UNIPLEX (Reductionist)

Mind = Brain = Computation

- Single layer of reality
- Psychology focused on neuroscience
- Humans reducible to mechanism
- Therapy = information processing

The Ontological Flattening Loop:
Humans reduced ↓, machines inflated ↑

MULTIPLEX (Integrative)

Body + Psyche + Spirit

- Multiple layers *(material, psychological, spiritual)*
- The heart (qalb) as seat of consciousness
- Humans as irreducibly complex
- Healing requires meaning-making

Both-And Multiplex Logic:
AI genuinely useful AND genuinely limited

Can a system without interiority guide beings toward interiority?

Basis of Multiplex Psychology & Mental Health

HORIZONTAL

BOTH-AND

- Openness to perspectives
- Polarity navigation
- Both-And Thinking
- Access AND quality
- Innovation AND safety

VERTICAL

VALUE-ORDERED

- Openness to transcendence
- Higher guides lower
- Interior orders exterior
- Dignity over efficiency
- Spirit over mechanism

SYSTEMIC

DYNAMIC/ SYSTEMIC

- Openness to emergence
- Systemic not linear thinking
- Feedback and adaptation
- Continuous navigation
- Divergent, not convergent

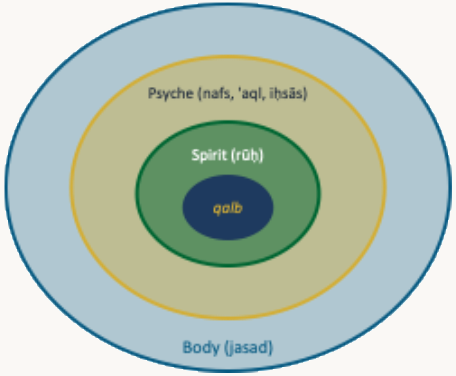
Five Anchor Principles (Maqāṣid) for AI Integration:

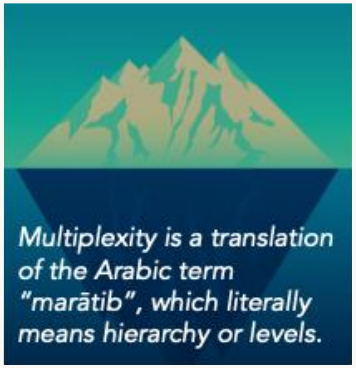
Human Dignity	Protection of Intellect	Benefit & Prevention of Harm	Human Accountability	Cultivation of Balance

Traditional Islamically Integrated Psychotherapy:

Clinical Embodiment of Multiplex Health

The Multiplex Psyche





Health and flourishing has a metaphysical basis.

Qadir, J., Qoronfleh, M. W., and Senturk, R. (2025). *Toward multiplex health: Integrating complexity, normativity, and open science*. Frontiers in Psychology, 16:1603474.c

AI-Human Task Division (Religious OCD Case)

QRDI-Funded ANMR Project: Integrating GenAI with TIIP for Religious OCD

Key: TIIP is relationship-embedded practice requiring human presence, and cannot be reduced entirely to information processing or algorithms

AI Role

- Psychoeducation
- Track anxiety patterns
- ERP prompts
- Progress monitoring
- Between-session support

Human Role

- Piety vs. pathology
- Fear → mercy reframe
- Ṣuḥba (companionship)
- Crisis response
- Tazkiyah (purification)

Seeking Your Wisdom

You are the experts in the territory we are mapping

Questions for Discussion:

- What should be the ideal division of tasks between AI and the therapist?
- How do we preserve *ṣuḥba* (spiritual companionship) in a digital age?
- What are the red lines that AI should never cross in Islamic psychotherapy?
- How can culturally-grounded frameworks like TIIP inform global AI governance?

"The question is not 'Can AI do therapy?' but 'What does our healing require?'"

Thank you!

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